

Puncture Wound - Female (Adult)

Algorithm 1234 | Adult (18 years and older) | Gender at birth: Female

Definition

- Safety-first assessment of a penetrating or puncture wound, including an embedded object, bite, needle or high-pressure injection injury, with anatomy, neurovascular status, age, and pregnancy considered.
- Adult patient.
- Female.

Initial assessment

1. [OPEN_TEXT] What caused the wound, when did it happen, and was it a bite, used needle, injection injury, high-pressure tool, dirty object, or intentional injury?
2. [LOCATION] Where is the wound, how deep may it be, and is any object or debris still embedded?
3. [OPEN_TEXT] Is bleeding controlled, and beyond the wound is the limb warm and normally coloured with normal feeling, movement, and pulse if trained to check?
4. [OPEN_TEXT] Does the wound involve the eye, head, neck, chest, abdomen, groin, genitals, hand, foot, or a joint?
5. [OPEN_TEXT] Record age, pregnancy, immune problems, diabetes, medicines affecting bleeding, tetanus vaccination history, and any increasing pain, redness, swelling, pus, fever, or illness.
6. [OPEN_TEXT] For an animal exposure, what animal was involved, where did it occur, was skin broken or saliva introduced, and is the animal available for official assessment? Do not try to capture it.
7. [YES_NO] If the patient is under 18, can they speak privately now, is the accompanying caregiver considered safe, and is there an approved safe callback method if contact is lost?

Triage and redirect questions

1. Redirect → Cuts and Lacerations

Is this an open cut or laceration rather than a narrow puncture wound?

An open cut or laceration belongs in the Cuts and Lacerations pathway.

1. Disposition level 100

If the patient is under 18, is harm occurring now, is there immediate danger from an accompanying or suspected caregiver, could the child be returned now to an unsafe person or place, or did contact end while an immediate threat remained unresolved?

Immediate danger, ongoing harm, an imminent unsafe return, or loss of contact during an unresolved immediate threat requires emergency action. Qatar 999 is the verified emergency route. Exact child-protection reporting, consent, documentation, and information-sharing requirements are local approval required.

2. Disposition level 78

If the patient is under 18 and there is no confirmed immediate danger, are they unable to speak privately, is caregiver safety uncertain, or is there another safeguarding concern requiring trained assessment?

An inability to obtain a private history, uncertainty about caregiver safety, or another safeguarding concern needs prompt trained assessment without automatically treating it as a confirmed emergency. The exact Qatar safeguarding pathway requires local approval.

3. Disposition level 100

Is bleeding heavy, spurting, or not controlled with firm pressure; is the person faint, pale, confused, or short of breath; is there a penetrating injury to the eye, neck, chest, abdomen, or groin; is a large or deeply embedded object present; is the limb beyond the wound pale, cold, pulseless, numb, or unable to move; or was this a high-pressure injection or injection of an unknown substance?

Uncontrolled bleeding, shock, critical-site penetration, major impalement, neurovascular compromise, and high-pressure injection can be limb- or life-threatening and require immediate emergency response.

3. Disposition level 70

If no emergency feature is present, is this an animal or human bite, used-needle injury, dirty or deep puncture, retained debris, delayed presentation, wound to a hand, foot, joint, genitals, or near a tendon; are infection signs developing; or is the patient a young child, pregnant, immunocompromised, diabetic, or unsure of tetanus protection?

These wounds need prompt in-person assessment for cleaning, structural injury, infection, blood-borne-virus exposure, and clinician-led tetanus or rabies risk management. The exact Qatar non-emergency service and timeframe are governance-required.

3. Disposition level 15

Is this an adult with a small, shallow, clean, recent non-bite and non-injection puncture, normal feeling and movement, controlled bleeding, no retained material or high-risk condition, and tetanus status already confirmed current by an approved local pathway?

Only a narrowly defined low-risk wound may enter home care. If any criterion is uncertain, use the governance-approved in-person route rather than self-care.

Advice

Immediate child safeguarding danger

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: harm occurring now or immediate danger; imminent return to an unsafe person or place; contact lost while an immediate threat remains unresolved.

Urgent private child safeguarding assessment

- Keep questions neutral and seek a private conversation only when it is safe. Do not disclose sensitive information to a caregiver whose safety is uncertain, do not confront or alert a suspected unsafe person, and do not promise confidentiality. Arrange prompt in-person assessment through the governance-approved Qatar child safeguarding pathway and record only an approved safe callback method. If immediate danger emerges or contact is lost during an unresolved immediate threat, use the emergency branch and call Qatar 999. Exact reporting, consent, documentation, and information-sharing rules are local approval required.
- Call Back If: private history cannot be obtained safely; caregiver safety remains uncertain; new immediate danger or loss of contact.

Emergency penetrating-wound precautions

- Call Qatar 999 now. Do not remove or push on an embedded object; stabilize it with padding and apply firm pressure around it if bleeding. For bleeding without an object, use firm direct pressure with a clean dressing. Keep the person still and warm, give nothing by mouth, and do not allow self-driving. Follow the 999 call-handler's instructions. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
- Call Back If: bleeding will not stop; faintness, confusion, breathing difficulty, or a pale cold limb.

Prompt in-person puncture-wound review

- Use the Qatar governance-approved in-person service and timeframe; the exact non-emergency route is not defined by this UAT protocol. Gently rinse visible contamination with clean running water, but do not probe, scrub deeply, close, or remove an embedded object. A clinician must assess cleaning, structural injury, infection, blood-borne-virus exposure, and whether tetanus vaccination or immunoglobulin, rabies prevention, or antibiotics are indicated. Pregnancy and pediatric medication choices require clinician or pharmacist confirmation.
- Call Back If: increasing redness, swelling, pain, pus, fever, red streaking, numbness, or reduced movement; bleeding, breathing difficulty, faintness, or limb colour/temperature change.

Home first aid for a strictly low-risk puncture

- Wash hands, control minor bleeding with a clean dressing, rinse the shallow wound with clean running water, pat the surrounding skin dry, and apply a sterile non-adherent dressing. Do not use bleach, hydrogen peroxide, or deep probing. Keep it clean and reassess regularly; seek the governance-approved in-person service if any eligibility criterion becomes uncertain.
- Call Back If: increasing pain, redness, warmth, swelling, pus, fever, red streaking, numbness, or reduced movement; the wound does not heal as expected.

First aid

- Call Qatar 999 now when immediate danger or ongoing harm is suspected. If contact continues, confirm the child's current location and a safe way to communicate without alerting or confronting a suspected unsafe caregiver. If contact is lost, use only the approved safe callback method and follow the governance-approved emergency escalation process. Do not promise confidentiality. Exact Qatar child-protection reporting, consent, documentation, and information-sharing rules are local approval required. Call Qatar 999 now, follow the call-taker's ambulance and emergency-transport instructions, and do not self-drive. Do not delay emergency care.
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References

- NHS.UK, "Cuts and grazes", <https://www.nhs.uk/conditions/cuts-and-grazes/> (page last reviewed 02 April 2026)
- US CDC, "Clinical Guidance for Wound Management to Prevent Tetanus", <https://www.cdc.gov/tetanus/hcp/clinical-guidance/> (accessed 25 July 2026)

- WHO, "Rabies", <https://www.who.int/news-room/fact-sheets/detail/rabies> (accessed 25 July 2026)
- Qatar Ministry of Public Health, "Healthcare Services in Qatar", <https://sportandhealth.moph.gov.qa/EN/faninfo/Pages/HealthcareServicesInQatar.aspx> (999 ambulance access; accessed 25 July 2026)

Provenance

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